Conduct Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Conduct disorder (CD) is a repetitive, persistent pattern in which a child or adolescent violates the basic rights of others or major age-appropriate rules: aggression toward people or animals, destruction of property, deceitfulness or theft, and serious rule-breaking (American Psychiatric Association [APA], 2022). It is the more serious relative of oppositional defiant disorder (ODD) in DSM-5-TR's "Disruptive, Impulse-Control, and Conduct Disorders" chapter.

How common: CD affects roughly 3% of school-aged children and is about twice as common in boys as in girls (Fairchild et al., 2019). Together, ODD and CD affect about 5.7% of children and adolescents worldwide (Polanczyk et al., 2015).

Why it matters: CD is highly impairing, often co-occurs with ADHD, and can continue into antisocial personality disorder in adulthood (Fairchild et al., 2019). In a long-term community study, CD mainly predicted later behavior problems, while ODD more often predicted later anxiety and depression (Rowe et al., 2010).

Counselor framing: behind most conduct problems are a young person and a family under strain. The goal is to change the child's environment and relationships, not only the child.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. At least **3 of 15** behaviors in the past 12 months, with at least 1 in the past 6 months, across four groups:

- **Aggression to people and animals:** bullying or threatening; starting fights; using a weapon; physical cruelty to people or to animals; stealing while confronting a victim; forcing someone into sexual activity.
- **Destruction of property:** deliberate fire-setting; deliberately destroying others' property.
- **Deceitfulness or theft:** breaking into a house, building, or car; lying to obtain goods or favors or to avoid obligations ("conning"); stealing without confronting a victim, such as shoplifting.
- **Serious rule violations:** staying out at night against parents' rules beginning before age 13; running away overnight at least twice; frequent truancy beginning before age 13.
- **Specifiers:** childhood-onset (at least one symptom before age 10), adolescent-onset, or unspecified onset; mild, moderate, or severe; and **"with limited prosocial emotions,"** meaning lack of remorse or guilt, callous lack of empathy, unconcern about performance, and shallow emotion.

The callous-unemotional subgroup. Youth with conduct problems and high callous-unemotional traits are at greater risk for severe and persistent antisocial outcomes. They respond less well to typical treatment but can respond to intensive approaches tailored to their emotional and thinking style (Frick et al., 2014).

When counselors must refer (general clinical consensus): CD needs a team. Refer for psychiatric evaluation when there is violence, weapon use, fire-setting, cruelty to animals, sexual aggression, substance use, self-harm, or co-occurring ADHD, depression, or learning problems. Assess safety for others, and follow mandated-reporting rules when you suspect abuse.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in conduct disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
APA Clinician-Rated Severity of Conduct Disorder	Rating conduct disorder severity.	Clinician-rated, brief	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures
ICU (Inventory of Callous-Unemotional Traits)	Callous-unemotional traits (the "limited prosocial emotions" specifier).	Parent, teacher, and self-report, 24 items	No (free)	None beyond general clinical training	Frick lab, LSU

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
SDQ (Strengths and Difficulties Questionnaire)	Brief screen of conduct, emotional, attention, and peer problems plus strengths.	Parent, teacher, and self-report, 25 items	No (free paper version)	None beyond general clinical training	SDQ website
NICHQ Vanderbilt Assessment Scales	Parent and teacher ratings of ADHD, ODD, conduct, anxiety, and depression symptoms, ages 6 to 12.	Parent and teacher forms, initial and follow-up	No (free)	None beyond general clinical training	NICHQ (PDF)
BASC-3 (Behavior Assessment System for Children)	Broad behavioral and emotional problems and strengths, ages 2 to 21 (college self-report to 25).	Parent, teacher, and self-report forms	Yes	★ Level B	Pearson
Conners 4	ADHD and related problems, ages 6 to 18.	Parent, teacher, and self-report forms (full, short, 12-item index)	Yes	★ Level B	MHS
Hare PCL:YV (Psychopathy Checklist: Youth Version)	Psychopathic traits in youth ages 12 to 18, usually in forensic settings.	Clinician rating from interview and records	Yes	★ Level C plus specialized training	MHS

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: for children, behavioral parent training; for adolescents, intensive family-based and multicomponent treatment combined with cognitive-behavioral skills. Medication can help aggression, mainly when ADHD is present.

Evidence overview (honest effect sizes):

- **Children (12 and under):** group parent behavior therapy and individual parent behavior therapy with the child taking part are "well-established"; 13 other treatments are "probably efficacious" (Kaminski & Claussen, 2017).
- **Adolescents:** reviewing 86 papers on 50 treatment programs, researchers rated two multicomponent treatments (combining family, behavioral, and cognitive-behavioral strategies) as "well-established," and two other multicomponent and two CBT programs as "probably efficacious" (McCart & Sheidow, 2016).
- **Multisystemic Therapy (MST),** which works with the family, school, peers, and neighborhood: across 22 studies of 4,066 youths, MST produced small but significant effects (van der Stouwe et al., 2014).
- **CBT in ordinary clinics:** across 51 studies (5,295 patients), CBT-based treatment for externalizing disorders in routine care led to large within-group improvement for CD and ODD (g = 0.98), with 48% remitting (Riise et al., 2021).
- **Medication (referral notes):** with co-occurring ADHD, stimulants had a moderate-to-large effect on conduct problems and aggression (high-quality evidence) (Pringsheim et al., 2015a). Among antipsychotics and mood stabilizers, only risperidone showed a moderate-to-large effect on aggression with moderate-quality evidence; the others were not well supported (Pringsheim et al., 2015b).

Cognitive-behavioral work with CD: how it is used (McCart & Sheidow, 2016; Riise et al., 2021):

- **Work with the system:** parents set clear supervision, rules, and consequences; schools and, when needed, courts coordinate.
- **Identify and challenge hostile and entitled thinking:** "He looked at me, so he wanted a fight," "If I want it, I should have it," "Rules are for suckers," "Nobody cares about me, so why should I care about anyone?"
- **Build replacement skills:** anger control, problem solving, seeing the other person's point of view, and rehearsing what to do instead of fighting or stealing.

5. Religion, spirituality, and Christian integration: what the evidence shows

Religious involvement and delinquency. Across 60 studies, religion had a moderate deterrent effect on criminal behavior (Baier & Wright, 2001). A later meta-analysis of 62 studies (193,656 adolescents) found that religious attitudes and church attendance were consistently linked to **less** alcohol use, drug use, and delinquency (correlations −.16 to −.22) (Kelly et al., 2015). These are correlations; they show association, not cause.

Faith-based programs. For adult offenders in Minnesota (732 released prisoners), a faith-based reentry program with mentoring inside and outside prison was linked to significantly lower rearrest, reconviction, and reincarceration; the authors credit its use of evidence-based cognitive-behavioral methods and its mentoring (Duwe & King, 2013). We found **no trial of a Christian-integrated treatment for youth with conduct disorder** (literature search, November 1, 2026).

Broader evidence. Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. Parent training, family-based multicomponent treatment, and CBT are the evidence base. Faith integration supports them; it does not replace them.
2. Religious involvement is consistently linked to less delinquency, and mentoring-rich faith-based programs show promise, but most of this evidence comes from observational studies and adults.
3. No randomized trial of Christian-integrated treatment for conduct disorder exists. Use faith as a source of relationships, structure, and hope alongside proven care.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within family-based, cognitive-behavioral care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for hostile, entitled, and hopeless beliefs: "Everybody's out to get me," "I deserve what I take," "I'm just a bad kid; God gave up on me." Listen to parents too: "This child is beyond hope" (Frick et al., 2014).
- **Challenge (clinical).** Examine the evidence for hostile assumptions in specific incidents, weigh the real costs of fighting and stealing, and build empathy step by step, using rewards for youth with low empathy (McCart & Sheidow, 2016). **Possible cognitive errors** to watch for in youth and in parents, with their definitions from the Belief Counseling Cognitive Errors Checklist and the symptoms they tend to feed:
  - **Jumping to conclusions** — deciding without the facts. *Feeds:* fighting and hostility. *Sounds like:* "He bumped me on purpose."
  - **Mind reading** — assuming I know what others think. *Feeds:* hostility toward adults. *Sounds like:* "The teacher hates me."
  - **Blaming others** — others are responsible for how I feel. *Feeds:* lack of remorse. *Sounds like:* "It's his fault I hit him."
  - **Unrealistic expectations** — expecting too much of myself or others. *Feeds:* stealing and entitlement. *Sounds like:* "I deserve what I take."
  - **Global labeling** — one mistake becomes who I am. *Feeds:* hopelessness and more rule-breaking. *Sounds like:* "I'm just a bad kid."
  - **Fortune telling** — predicting a bad outcome. *Feeds:* parents giving up. *Sounds like:* "This child is beyond hope."
- **Questions to dispute the belief.** Have the parent (or an older child) choose questions from the Questioning False Beliefs form that "speak to" the belief. Selected questions that fit the errors above:
  - "Are there other ways of looking at this situation?" *For:* jumping to conclusions.
  - "What's the actual evidence for my belief?" *For:* mind reading.
  - "What would Christ do in this situation?" *For:* blaming others.
  - "What are some ways that I could make my situation worse?" *For:* entitlement and weighing the costs.
  - "Doesn't everyone have good points and bad points?" and "Has God given up hope that things will get better for me?" *For:* global labeling and "God gave up on me".
  - "Am I completely destined to stay this way my whole life?" *For:* a parent's fortune telling.
- **Replace (pastoral).** Offer Scripture of responsibility, restitution, and a new start:
  - "He who has been stealing must steal no longer, but must labor, doing good with his own hands, that he may have something to share with the one in need." (Ephesians 4:28). Replacing taking with working and giving.
  - "Do not be deceived: God is not to be mocked. Whatever a man sows, he will reap in return." (Galatians 6:7). Actions have consequences.
  - "Therefore if anyone is in Christ, he is a new creation. The old has passed away. Behold, the new has come!" (2 Corinthians 5:17). No young person is beyond hope.
  - "I will give you a new heart and put a new spirit within you; I will remove your heart of stone and give you a heart of flesh." (Ezekiel 36:26). For the hardened heart.

- **Faith community (clinical).** Offer steady adult mentors, supervised youth activities, and practical support for parents. Mentoring was a key part of the faith-based program above (Duwe & King, 2013). Keep other children safe with clear supervision.
- **Ministers:** never minimize violence, cruelty, or sexual aggression; report abuse; and refer to qualified clinicians rather than handling CD alone.

7. Quick classroom takeaways

- CD is a pattern of violating others' rights: 3 of 15 behaviors across aggression, destruction, deceit or theft, and serious rule-breaking. Note onset age and "limited prosocial emotions."
- Parent training for children; intensive family-based and CBT treatment for teens; stimulants or risperidone for aggression in some cases.
- Replace "I'm just a bad kid" with responsibility and the hope of a new heart, and surround the young person with steady mentors.
- Measure callous-unemotional traits (free ICU) to identify youth who need intensive, tailored treatment.

Sources note

All 15 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 15 references and DOIs verified via Crossref, OpenAlex, and PubMed/Europe PMC records (Sunday, Nov 1, 2026). Effect sizes as reported in the cited sources.